

Q1 2023 Earnings Call

Company Participants

- Carrie Strom, Senior Vice-President and President Allergan Aesthetics
- Jeff Stewart, Executive Vice President, Chief Commercial Officer
- Liz Shea, Senior Vice-President Investor Relations
- Rick Gonzalez, Chairman of the Board and Chief Executive Officer
- Rob Michael, Vice Chairman and President
- Roopal Thakkar, Senior Vice President of Development and Regulatory Affairs and Chief Medical Officer
- Scott Reents, Executive vice-president and chief financial officer
- Tom Hudson, Senior Vice President, R&D, and Chief Scientific Officer

Other Participants

- Alice Nettleton, Analyst, Wolfe Research
- Carson, Credit Suisse
- Carter Gould, Analyst, Barclays
- Chris Schott, JP Morgan
- Evan Seigerman, Analyst, BMO Capital Markets
- Mohit Bansal, Wells Fargo
- Steve Scala, Cowen
- Susan, Bank of America
- Terence Flynn, Analyst, Morgan Stanley
- Vamil Divan, Analyst, Guggenheim Securities

Presentation

Operator

Good morning, and thank you for standing by. Welcome to the AbbVie First Quarter 2023 Earnings Conference Call. (Operator Instructions) I would now like to introduce Ms. Liz Shea, Senior Vice-President, Investor Relations.

Liz Shea {[BIO 15134332 <GO>](#)}

Good morning, and thanks for joining us. Also on the call with me today are; Rick Gonzalez, Chairman of the Board and Chief Executive Officer, Rob Michael Vice-Chairman and President, Jeff Stewart, Executive Vice-President and Chief Commercial Officer, Scott Reents, Executive Vice-President and Chief Financial Officer, Carrie Strom, Senior Vice-President and President Allergan Aesthetics, and Tom Hudson, Senior Vice-President, R&D and Chief Scientific Officer. Joining us for the Q&A portion of the call is Roopal Thakkar Senior Vice-President, Development and Regulatory Affairs and Chief Medical Officer.

Before we get started, I'll note that some statements we make today may be considered forward-looking statements based on our current expectations. AbbVie cautions that these forward-looking statements are subject to risks and uncertainties that may cause actual results to differ materially from those indicated in our forward-looking statements. Additional information about these risks and uncertainties is included in our SEC filings. AbbVie undertakes no obligation to update these forward-looking statements, except as required by law.

On today's conference call, non-GAAP financial measures will be used to help investors understand Abbvie's business performance. These non-GAAP financial measures are reconciled with comparable GAAP financial measures in our earnings release and regulatory filings from today, which can be found on our website. Following our prepared remarks, we'll take your questions.

So with that, I'll turn the call over to Rick.

Rick Gonzalez {[BIO 1973579 <GO>](#)}

Thank you, Liz. Good morning, everyone, and thank you for joining us today. I'm extremely pleased with our start to 2023. With first quarter total revenues and adjusted earnings per share both exceeding our expectations. This performance was driven by double-digit sales growth from several key products including Skyrizi, Rinvoq, Venclexta and Vraylar, positive momentum from our Aesthetics business with strong results internationally and stabilizing consumer trends in the US.

And in-line performance from US Humira our bio-similar erosion is tracking as expected with much of an impact driven by price. Since our inception, we have successfully created a well-diversified portfolio with multiple growth platforms in highly-attractive markets including immunology, hematological oncology, neuroscience and Aesthetics.

Our commercial execution, including the launch of new products and expanded indications, has been outstanding, especially across Skyrizi and Rinvoq and recently with Vraylar and MDD. Each of these assets are expected to contribute significant revenue growth over the decade. The breadth and the depth of our R&D pipeline also supports our long-term growth outlook and we anticipate numerous important pipeline milestones over the next two years.

In summary, we're one quarter into the US biosimilar event for Humira and we're managing the erosion well. Most importantly, our growth platform is demonstrating strong performance exceeding our expectations. We're executing well across all aspects of our business and see numerous opportunities for our diverse portfolio to drive long-term growth.

With that, I'll turn the call over to Rob for additional comments on our business performance. Rob.

Rob Michael {[BIO 19541407 <GO>](#)}

Thank you, Rick. We're off to an excellent start in 2023 with each of our five key therapeutic areas, meeting or exceeding our first-quarter expectations, a testament to the strength of our broad portfolio. We delivered adjusted earnings per share of \$2.46 which is \$0.10 above our guidance midpoint. Total net revenues were \$12.2 billion, approximately \$400 million ahead of our expectations.

First-quarter results include continued robust performance from Skyrizi and Rinvoq; which remain on track to contribute more than \$11 billion in combined sales this year. Growth rates in the first quarter for both products are consistent with our full-year expectations. Skyrizi and Rinvoq are demonstrating momentum across all approved indications and we expect to round out their opportunities in IBD later this year. This includes Rinvoq's anticipated US approval in Crohn's disease as well as Skyrizi's European launch in Crohn's and its global regulatory submission in UC.

We are also performing exceptionally well in neuroscience. Total net revenues this quarter were nearly \$200 million above our guidance. With Vraylar sales accelerating following MDD approval and migraine delivering strong growth as a result, we'll be increasing our full-year outlook for neuroscience. Aesthetics is also performing better than expected. We are seeing positive recovery trends in China and some stability in the US market where we are closely monitoring

several economic indicators that correlate with aesthetics procedures including consumer confidence, personal consumption and Google searches.

Although it's still early in the year these positive trends, especially across our international markets give us the confidence to increase our full-year outlook for aesthetics as well. This continues to be an under penetrated market with significant growth potential. Based on our robust performance this quarter and the continued strong outlook for our business, we are raising our full-year adjusted earnings per share guidance by \$0.10 and now expect adjusted earnings per share between \$10.72 and \$11.12.

In closing, I'm extremely pleased with the performance of our diverse portfolio. We're off to a strong start to the year, which further reinforces our confidence in the long-term outlook of the business.

With that, I will turn the call over to Jeff for additional comments on our commercial highlights. Jeff.

Jeff Stewart {BIO 19485431 <GO>}

Thank you, Rob. I'm very pleased with the strong commercial execution across our therapeutic portfolio. Immunology delivered total revenues of approximately \$5.6 billion with continued robust double-digit growth from Skyrizi and Rinvoq. Skyrizi global sales were nearly \$1.4 billion reflecting operational growth of more than 46% despite retail inventory destocking in the quarter. Skyrizi is the clear market leader in the US biologic psoriasis market with a total prescription share now at 30%.

In psoriasis, Skyrizi has set a very high bar relative to other therapies on the market or in development with differentiated attributes across the categories that physicians and patients deem most important. This includes the rapid onset of action after the first dose, nearly complete skin clearance with multi-fold higher rates on PASI 90 and PASI 100. High durability of response, which we have demonstrated can increase over time. As well as quarterly dosing for maintenance therapy, a convenient alternative to daily oral or more frequently administered injectables.

With a nearly 50% US in-play share of new and switching patients, there is substantial room for Skyrizi's continued growth in psoriasis. This best-in-class profile is supporting strong momentum now in psoriatic arthritis with Skyrizi achieving an in-play biologic share of roughly 20% in the US dermatology segment.

Skyrizi is also being co-positioned with Rinvoq in the US room segment in PSA where we're seeing increasing utilization among rheumatologists as well. Globally Skyrizi has achieved in-play psoriatic disease leadership in more than 25 countries and total market-share leadership in nearly 20 of those key markets.

In Crohn's disease, we are seeing very fast adoption of Skyrizi in the US with a total in-play patient share at approximately 20%, second only to Stelara. Feedback from gastroenterologists is very positive, especially as it relates to Skyrizi's novel dosing and overall clinical profile. We see strong uptake in Japan in Canada as well with the European launch forthcoming.

We also recently reported strong induction data for Skyrizi in ulcerative colitis, which Tom will discuss momentarily. Based on the results of that trial. It is increasingly clear that Skyrizi represents a differentiated asset across inflammatory bowel disease and we look forward to bringing this potential new indication to physicians and patients next year.

Turning now to Rinvoq which delivered global sales of \$686 million reflecting operational growth of more than 50% despite similar retail inventory destocking in the quarter. I'm very pleased with the performance in rheumatology with total prescriptions increasing across each of the four approved indications. Atopic dermatitis is also tracking in line with our expectations. We continue to see market-share momentum globally, including in-play patient share increasing to approximately 17% in the US.

We are very excited about the growth potential in gastroenterology. Rinvoq has set a high bar for efficacy in both ulcerative colitis and Crohn's disease, demonstrating strong rates of remission and endoscopic improvement. We're seeing very strong momentum in UC where adoption has been robust. Rinvoq is now achieving a 23% in-play share in the US second-line plus setting reflecting an impressive ramp since our launch in UC less than one year ago.

This accelerated adoption among gastroenterologists is very encouraging for Rinvoq's pending outlook in Crohn's. We are currently launching this indication in the EU, a geography where Rinvoq is the only JAK approved to treat both IBD conditions and we remain on track for CD approval and commercialization in the US later this quarter with broad formulary access anticipated to ramp quickly over the back half of this year. So we see inflammatory bowel disease continues to be an area of high unmet need, having two novel therapies in IBD with Skyrizi and Rinvoq that each deliver differentiated levels of efficacy as an important step forward for patients. And with these two complementary assets, we are very well-positioned to compete against other oral or biological agents.

Global Humira sales were approximately \$3.5 billion, down 24.3% on an operational basis due to biosimilar competition. Erosion in the US remains in line with our expectations with most of the impact driven by price.

Turning now to hematologic oncology, where total revenues were \$1.4 billion with continued pressure on Imbruvica, partially offset by robust double-digit growth with Venclexta. Imbruvica global revenues were \$878 million, down 25.2% due to increasing competition and the cumulative impact of a suppressed market. Venclexta global sales were \$538 million, up 17.5% on an operational basis with strong momentum across both AML and CLL.

In neuroscience, revenues were approximately \$1.7 billion, up 15% on an operational basis. Vraylar is performing exceptionally well. Sales of \$561 million were up 31.3% on an operational basis above our expectations. We're very pleased with the AMDD label and the launch, which has resulted in a significant uplift in total new prescriptions for Vraylar.

With a dedicated sales force that calls on both psychiatrists and primary care, as well as ramping DTC promotion, we see an opportunity for accelerated growth across all approved indications and we will be raising our full-year guidance for Vraylar accordingly.

Within migraine, we remain uniquely positioned with a portfolio to support complete migraine freedom. Our leading oral CGRP therapies contributed \$218 million in combined sales this quarter, reflecting growth of more than 45% as we continue to see strong prescription demand for both Ubrelvy and Qulipta. We recently expanded the label for Qulipta, which is now uniquely positioned as the only oral CGRP available as a preventative treatment for patients with both chronic and episodic migraine, further strengthening our competitive profile.

Lastly, total botox therapeutic sales were \$719 million, reflecting strong performance in chronic migraine, as well as other approved indications. So overall, I'm extremely pleased with the performance across the therapeutic portfolio.

And with that, I'll turn the call over to Carrie for additional comments on aesthetics. Carrie.

Carrie Strom {[BIO](#) [20606775](#) <GO>}

Thank you, Jeff. First-quarter global aesthetics sales were approximately \$1.3 billion, which came in ahead of our guidance primarily due to a faster reopening in China as well as a slightly stronger economy in the US versus our planning assumptions. In the US aesthetics sales were \$777 million, down 8.1% as we continue to see softness in aesthetic procedures related to inflationary dynamics.

As a reminder, we saw a very robust performance for our US performance in the first quarter of 2022, which created a difficult comparison for growth in the first quarter of this year. US botox cosmetic sales were \$409 million, down slightly on a year-over-year basis. We continue to see a lesser impact from inflationary dynamics on botox cosmetic compared to other areas of our aesthetics portfolio due to its relatively lower price point and large install-base of loyal repeat consumers. The US cosmetic toxin market was down low-single digits in the first quarter on a year-over-year basis.

Botox cosmetic continues to be the clear market leader and its share of the US toxin market remains stable. Sales for our US Juvederm collection were down 18% as our dermal filler portfolio continues to be impacted by inflationary pressure on consumer spending. US filler market was down nearly 20% in the quarter on a year-over-year basis, due to the persistent inflationary environment.

Our Juvederm collection remains the clear market leader and share was stable in the quarter. The economic pressure on our US dermal filler business was partially offset in the quarter by strong initial uptake for our recently launched Evolus filler, which is approved for the improvement of jaw line definition. We expect Evolus combined with the upcoming launch of our skin quality injectables skin ViiV to support long-term growth for our filler portfolio in the US.

While the aesthetics category in the US continues to be challenged due to the fast economy, the key external economic metrics that we track have remained relatively consistent with year-end 2022 levels.

Our international Aesthetics portfolio continues to demonstrate robust growth with strong performance in Japan, which is rapidly growing and China, which is recovering faster than expected. Sales from our international aesthetics portfolio were \$523 million, up 7.8% on an operational basis. International botox cosmetic sales grew approximately 17.5% operationally and international Juvederm sales were down approximately 1.4% on an operational basis.

China, which is our second-largest market, was negatively impacted by COVID in January and February, but experienced a sharp recovery in March. We expect this level of activity to be sustained throughout the remainder of the year. Recall, our original guidance assumed we would not reach a full recovery until the second half of this year. And in Japan, which is an underdeveloped market and proving to be very responsive to promotion, we continue to make significant investments in injector training, our field force and consumer education.

Overall, we're pleased with how our team has been executing through this dynamic environment, and remain encouraged by improving trends internationally and stabilization across our US portfolio. These positive trends and continued strong momentum gives us the confidence to increase the full-year outlook for our aesthetics business.

Longer-term, we remain extremely confident in our ability to grow the aesthetics business and continue to expect to achieve total sales of more than \$9 billion by the end of this decade. Aesthetics continues to be an extremely attractive under penetrated market and our proven ability to drive consumer demand and develop a strong base of loyal customers, as well as bring innovative new products to the market, will support robust growth over the long term.

With that, I'll turn the call over to Tom.

Tom Hudson {[BIO 15189431](#) <GO>}

Thank you, Carrie. We've continued to make very good progress with our pipeline to start this year. In immunology, we recently received European approval for Rinvoq in Crohn's disease, making it the first JAK inhibitor approved for this indication. We continue to anticipate FDA approval for Rinvoq in Crohn's disease next month.

We also recently announced positive top-line results from our phase III induction study for Skyrizi in ulcerative colitis which is a disease with unpredictable symptoms and frequent flares, making it challenging to manage. In our study, Skyrizi met the primary and all secondary endpoints, demonstrating a very strong impact on the disease as measured by clinical remission, clinical response and endoscopic improvement.

We're particularly pleased with Skyrizi's impressive performance on the more stringent measures in this trial with approximately 37% of Skyrizi-treated patients achieving endoscopic improvement compared to 12% of patients on placebo. This level of efficacy has the potential to position Skyrizi as a highly effective therapy, and we believe it will be a welcome new treatment option for physicians and patients once approved.

Detailed data from this induction study will be presented at a forthcoming medical meeting. We expect to see data from the phase III maintenance study in the second quarter with our regulatory submissions planned for the second half of the year.

In oncology, we continue to make good progress across all stages of our hematology and solid tumor pipeline. We remain on track for several important regulatory and clinical milestones this year including regulatory approval for Epcoritamab in relapsed/refractory large B-cell lymphoma. Phase III data from Venclexta's Quinova trial in relapsed/refractory multiple myeloma patients with the t(11;14) mutation and Navitoclax's TRANSFORM-I trial in frontline myelofibrosis.

And phase II data for Teliso-V in second line plus advanced non-squamous, non-small cell lung cancer, which has the potential to support a regulatory submission for accelerated approval. We're also beginning to see very encouraging data for our next-generation c-MET ADC which uses a more potent total payload than our Teliso-V ADC.

Based on the data we've seen to date for ABBV-400 in our phase I solid tumor BASKET study we plan to expand the program to earlier lines in colorectal cancer, as well as evaluate in other tumors where c-MET is expressed including pancreatic and liver cancer.

Moving to our neuroscience pipeline where we have recently received FDA approval for Qulipta as a preventive treatment for patients with chronic migraine, making it the only oral CGRP antagonist approved for prevention of both episodic and chronic migraine. In our phase III study Qulipta provided a significant reduction in migraine days as well as significant improvements in function and quality of life in patients with chronic migraine, a common and debilitating disease. As a highly effective oral treatment option we believe Qulipta will be well-positioned in a chronic migraine prevention market.

In Europe, we continue to anticipate an approval decision in the third quarter for Atogepant as a preventive treatment for patients with both chronic and episodic migraine.

Turning now to ABBV-951. We announced that we received a complete response letter for our regulatory application in the US. The FDA has not asked for additional efficacy or safety studies related to our drug device delivery system, but rather they have requested additional information regarding the pump as well as updates to instruction for use. We are working to generate the necessary information and we expect to respond to the CRL later this year with an FDA action anticipated in the first half of '24.

In international markets, we recently received approval for 951 in Japan and we continue to expect approval in Europe in the fourth quarter of this year. In our early-stage neuroscience pipeline, we recently began phase I studies of our selective D3 dopamine receptor agonist ABBV-932. Our experience with Vraylar has highlighted the potential clinical benefit of achieving D3 selectivity and we believe that a compound that more selectively engages the D3 dopamine receptor has the potential to provide enhanced efficacy.

Our program will initially focus on general anxiety disorder with the potential to expand to other neuropsychiatric disorders. The programs under our collaboration with Calico are also progressing well. We now have four assets in clinical trials including two PTPN2 and inhibitors in phase I in oncology. Our eIF2B activator for neurodegenerative diseases and an IGF-1 signaling pathway modulator that will be explored in aging-related diseases.

Our most advanced program is the eIF2B activator 7262. The first patient was recently enrolled in the Healey ALS platform trial a phase II/III study conducted by the Healey Center for ALS at Mass General. This trial is designed to evaluate multiple therapies simultaneously with the goal to accelerate the development of potential breakthrough treatments for ALS.

Now I'd like to provide a brief update on two earlier-stage programs in our therapeutic pipeline. In cystic fibrosis, we recently analyzed data from an ongoing proof-of-concept study evaluating our triple combination therapy. The results from this interim analysis did not meet our criteria for advancing and we are discontinuing our cystic fibrosis program.

We also recently reviewed interim data from our exploratory studies for ABBV-154 in PMR and Crohn's disease. Similar to results from the RA study while we observe efficacy with 154, we also observe some changes in biomarkers that are consistent with systemic steroid exposure at the higher doses. The benefit-risk profile does not sufficiently differentiate 154 from other available treatments. So based on the totality of the data across RA PMR and Crohn's disease studies, we will not be pursuing further development of this asset.

Now moving to our aesthetics pipeline. We recently saw data from our phase III studies for Botox in platysma prominence and masseter muscle prominence. Study for prominent neck muscles Botox met all primary and secondary endpoints demonstrating a significant reduction in the unwanted appearance of platysma prominence on the neck and jawline. This was the first of three Phase III studies in platysma prominence with data from the two remaining trials expected in the second half of the year followed by regulatory submission in the US, near the end of 2023.

Botox also performed very well in our study for prominent masseter muscles meeting the primary and all secondary endpoints in the trial. Our program is initially focused on China and other Asian markets as masseter prominence is common in Asian populations and there is significant unmet need for minimally-invasive treatment options.

Based on the results from this trial, we expect to submit our regulatory application in China in the second half of the year. Once approved, we anticipate high demand for Botox in this novel indication, which will help to further build our portfolio in the lower face segment.

So in summary, we continue to demonstrate significant progress across all stages of our pipeline and anticipate numerous important regulatory and clinical milestones throughout the remainder of 2023.

With that, I'll turn the call over to Scott.

Scott Reents {[BIO 21044531](#) <GO>}

Thank you, Tom. I will discuss our most recent financial results and guidance. Starting with our first-quarter results, we delivered strong top and bottom-line performance. We reported adjusted earnings per share of \$2.46 which is \$0.10 above our guidance midpoint. These results included \$0.08 unfavorable impact from acquired IPR&D expense.

Total net revenues were \$12.2 billion, \$400 million ahead of our guidance and down 8.3% on an operational basis, excluding a 1.4% unfavorable impact from foreign exchange. The adjusted operating margin ratio was 45% of sales. This includes adjusted gross margin of 84.2% of sales. Adjusted R&D investment of 13.6% of sales. Acquired IPR&D expense of 1.2% of sales and adjusted SG&A expense of 24.4% of sales. Net interest expense was \$454 million, the adjusted tax rate was 13.7%.

Turning to our financial outlook, we are raising our full-year adjusted earnings per share guidance to between \$10.72 and \$11.12. This earning-per-share guidance does not include an estimate for acquired IPR&D expense that may be incurred beyond the first quarter. We now expect net revenues of approximately \$52.4 billion, an increase of \$400 million. At current rates, we expect foreign exchange to have a modest unfavorable impact on full-year sales growth.

This guidance includes the following updated assumptions. We now expect Vraylar sales of approximately \$2.7 billion, an increase of \$200 million reflecting strong prescription growth following the MDD approval. And for aesthetics, we now expect global revenue of approximately \$5.3 billion reflecting the better-than-expected recovery in China and stable economic trends in the US.

Turning to the second quarter, we anticipate net revenues of approximately \$13.5 billion, which includes US Humira erosion of 27%. At current rates, we expect foreign exchange to have a 0.6% unfavorable impact on sales growth. We are forecasting an adjusted operating margin ratio of 48.5% of sales. We are modeling a non-GAAP tax rate of 15.4%. We expect adjusted earnings per share between \$2.90 and \$3. This guidance does not include acquired IPR&D expense that may be incurred in the quarter.

In closing, we're off to an excellent start to the year with strong performance across the portfolio and financial results ahead of our expectations.

With that, I'll turn the call-back over to Liz.

Liz Shea {[BIO 15134332](#) <GO>}

Thanks, Scott. We will now open the call for questions. In the interest of hearing from as many analysts as possible over the remainder of the call, we ask that you please limit your questions to one or two. Operator, first question please.

Questions And Answers

Operator

Thank you. Our first question comes from Terence Flynn with Morgan Stanley. Your line is open.

Q - Terence Flynn {[BIO 15030404 <GO>](#)}

Great, thanks so much for taking the question. Maybe two from me. Just on immunology, can you quantify the amount of destocking for both Skyrizi and Rinvoq in the quarter? I think Biogen on their call spoke to some tighter working capital requirements at wholesalers due to rising interest rates. So just wondering if you're seeing something similar here and just to want to be sure that it's not a pricing dynamic. And then, can you elaborate at all about your ALS program when we might see some data there. Thank you

A - Rob Michael {[BIO 19541407 <GO>](#)}

Hi, Terence, it's Rob. So on the retail inventory destocking we do typically see this in the first quarter. So it wasn't a complete surprise in fact that was factored into our guidance. We did beat our guidance in immunology for the quarter. The impact was high-single-digits, both for Skyrizi and Rinvoq in terms of absolute value, you're talking about around \$70 million for Skyrizi and \$30 million for Rinvoq.

A - Jeff Stewart {[BIO 19485431 <GO>](#)}

And it's Jeff, just to clarify on your wholesale comment. I think as Rob highlighted, it's in the retail. So it's in the specialty pharmacy channel. And you'll recall, as you know there's about 18 specialty pharmacies that basically distribute the I&I products and there are some big ones there. So it was not a wholesaler dynamic. It was a retail inventory dynamic, which again as these products get bigger, we do see and contemplate in our projections.

A - Rob Michael {[BIO 19541407 <GO>](#)}

And then, Terence, this is Rob, just to more broadly answer the question that I'm sure many investors have if you look at the growth in the quarter -- so overall, US demand was up just north of 60% for Rinvoq and Skyrizi. We saw very strong performance across all approved indications as Jeff highlighted. We did have a two partial offsets one being the retail inventory destock, which I've already quantified and then price was down high-single-digits. And that's really driven by rebate increases, which is typical when you see the type of volume increase and we saw Skyrizi sales up 80% last year. Rinvoq in the US up 40% last year.

So when you see that kind of volume growth, and coupled that with a number of indications, we had approved, we had five new indications for Rinvoq and two for Skyrizi. So it was something we were contemplating, it was factored into our guidance, but I don't know that the street fully appreciated that in terms of the first-quarter estimates that we put out there.

A - Tom Hudson {[BIO 15189431 <GO>](#)}

This is Tom for the ALS question. I'd say about two years, we have to recruit the patients on this about a year of follow-up that is needed. So I'd say late 24 early 25, all depends on the enrollment, which I understand is starting off on a good pace.

A - Liz Shea {[BIO 15134332 <GO>](#)}

Thanks, Terrence. Operator next questions, please.

Operator

Thank you. Our next question comes from Steve Scala with Cowen. Your line is open.

Q - Steve Scala {BIO 1505201 <GO>}

Thank you. A couple of questions. Some of your peers have called out co-pay resets early in the year that have led to more modest performances in products such as Dupixent and Cosentyx. AbbVie has been less vocal on this point, how much was that a factor or are Skyrizi and Rinvoq different in some way? And then secondly, despite the solid performance in Q1, the EPS guidance range continues to be very wide, what factors would have to play out for you to hit the high end of that and the low end of that? Thank you.

A - Jeff Stewart {BIO 19485431 <GO>}

Hi, Steve, it's Jeff and thanks for the question. I'll take the copay one. Look, I mean some of the peers are seeing the effects of the so-called maximize your programs or even some of the lingering accumulator programs which do sometimes as benefit designs are reset in the first quarter can apply some pressure. We don't see that. We've been managing that very tightly and we're not seeing any significant surge or creep in terms of that dynamic.

The dynamic is exactly what Rob had highlighted, which is the modest price based on the number of indications and how fast the volume is moving in this destocking event that we talked through. So copay is very stable for AbbVie.

A - Rob Michael {BIO 19541407 <GO>}

Steve, this is Rob. On your EPS range question we've typically given a \$0.20 range, last year we gave a \$0.40 range and the key driver of that is obviously the US Humira dynamics. As we see that play out, particularly in the second half, we would typically tighten that range. Now keep in mind that \$0.20 wide range represents about 1% of US Humira growth. So it's not as wide as you might think, but if we did widen that given the dynamics with US Humira and I think we'll be able to give you more color as we see those seven, nine biosimilars coming to market in the middle of the year, we'll have more clarity for you on the second-quarter call.

Q - Steve Scala {BIO 1505201 <GO>}

Thank you.

A - Liz Shea {BIO 15134332 <GO>}

Thanks, Steve. Operator, next question please.

Operator

Our next question comes from Chris Schott with JP Morgan. Your line is open.

Q - Chris Schott {BIO 6299911 <GO>}

Great, thanks so much. Just coming back to Skyrizi and Rinvoq. I think you mentioned high-single-digit price erosion beyond just those inventory changes. I guess, is that a reasonable assumption to think about price as we progress through the rest of this year. And then maybe longer-term, should we think about that level of price erosion as more like a one-time kind of issue this year,

given all the new indications and a more stable trend going forward? I'm trying to get my hands around the pricing a little bit more.

And my second question was on the aesthetics business. You obviously saw very strong 1Q. Can you just elaborate a little bit more on your confidence in the sustainability of these trends. I know the market has been a little bit bumpy, but it seems like the tone is that you're encouraged with the trends you're seeing. But just how much visibility do you have in terms of sustainability of the favorable international trends we're seeing right now? Thank you.

A - Rob Michael {BIO 19541407 <GO>}

Chris, this is Rob. So on your question on price, yeah, the way to think about 2023 price for the year for Skyrizi and Rinvoq will be down high-single-digits. Now, we wouldn't expect high-single-digits to be the rate going forward given a big driver was the number of new indications that we launched and so I would expect that to moderate over time.

A - Carrie Strom {BIO 20606775 <GO>}

Hi, this is Carrie. In terms of your question around the aesthetics market, there are two key assumptions for the 2023 planning. One was around the US economy and the other was around the recovery in China. When you think about the US economy, we look at Q1 and we see some favorability to our planning assumptions with the metrics that we track, which Rob mentioned include real personal consumption expenditure and Google.

So in January and February, we saw favorability there, although in March there are some potential signs of softening. So we continue to have a cautious outlook for our US business for the rest of the year. The way to think about market growth for the full year would be for toxins the market would be down low-to mid-single-digits until we lap the 2022 downturn, which happened around May and then after that, we would expect flat market growth for the rest of the year. So that's how we think about the US.

Our other -- our second-biggest market is China. And recall we had assumed that the aesthetics market in China would not fully recover until the second half of the year. Well, in actuality what we saw was although January was significantly impacted, in February, we started to see improvement and in March there was a really steep recovery that we do expect will sustain through the rest of the year.

And in other markets around the world. I guess in Canada and UK, we are seeing some high inflation impacting consumer demand, but the rest of Europe seems stable right now.

A - Liz Shea {BIO 15134332 <GO>}

Thank you, Chris. Operator, next question please.

Operator

Our next question comes from Mohit Bansal with Wells Fargo, your line is open.

Q - Mohit Bansal {BIO 18070890 <GO>}

Great, thanks for taking my question. I have one clarification and one question. So classification is regarding the price decline for I&I, you said that high-single-digit, but should we assume going forward, year-over-year, versus volumes, should we assume high-single-digit declines still going

forward for at least for rest of the quarter and then more stable pricing quarter-over-quarter. Tom, just that clarification.

And the question is regarding the I&I pipeline, so you have a leadership position with Skyrizi and Rinvoq and they are growing, but some of the pipeline products like 154 and 157 did not pan out as you were hoping they do. So what is -- how should we think about the pipeline strategy beyond these two products and can you even do M&A given this increased FTC scrutiny nowadays? Thank you.

A - Rick Gonzalez {BIO 1973579 <GO>}

This is Rick. I'll cover number two, and maybe just touch on number one. If you think about our pipeline, obviously, as we look at Skyrizi and Rinvoq, they clearly have restated the immunology market across most of the segments that we operate in. We view those assets as being able to drive strong growth through the early part of the next decade. Having said that, we're continuing to look for assets in areas where we believe there is still a significant opportunity to restate standard-of-care and we obviously explore as everyone in this industry does many different assets and different mechanisms to try to find those kinds of mechanism that will deliver that kind of performance.

It's interesting when you look at the 154 platform, it did exactly what we thought it would do from the standpoint of efficacy. But it did it in a way only at the highest dose and at that highest dose we did see some effects of steroids on some of the biomarkers. And based on the way we think regulators would look at the label for those kinds of products, we didn't believe that would be a competitive profile but the hypotheses certainly weren't around the mechanism. So we continue to look for opportunities. We have lots of runway here to be able to get to those, but we do desire to find some additional mechanisms that will be the follow-on products that should be introduced, hopefully near the end of this decade or early into the next decade as the next-generation immunology assets for AbbVie. And I feel good about the progress that we're making there. We're continuing to explore a number of other areas, and we're continuing to look both internally and externally at different assets that we can bring into the company to be able to do that

To your question of being able to bring assets into this market, we don't believe that we would be encumbered because immunology is a very crowded space from a competitive standpoint, and that's one of the most important criteria that you look at from an FTC standpoint. So we believe we have freedom to operate across most of those segments from an FTC standpoint as well.

And on price, maybe, Rob and I will tag team this one to make sure it's clear. It is common that when you go out and you add indications in this industry that when you negotiate those contracts to be able to get access it does require some level of price concession. I would say we're on the lower end of what you typically would have seen with the speed at which we got access for Skyrizi and Rinvoq for those indications and the breadth of that access.

And so we certainly would expect this year and last year, to be the areas where you saw the most significant amount of price because those are the years that we added the majority of the indications. You would expect that to moderate.

So then going forward the way to think about it it's only really driven by volume at that point and volume typically requires much more modest kinds of price as you go forward. It's not zero-price. You should have that expectation. But I would not have an expectation of high-single-digits going forward.

Anything you'd add, Rob?

A - Rob Michael {[BIO 19541407 <GO>](#)}

Just to answer the question, Mohit on the gating. Yeah, I think it's safe to assume that you will see high-single-digit price in each of the quarters this year.

Q - Mohit Bansal {[BIO 18070890 <GO>](#)}

Helpful. Thank you.

A - Liz Shea {[BIO 15134332 <GO>](#)}

Thank you, Mohit. Operator, next question please.

Operator

Our next question comes from Tim Anderson with Wolfe Research. Your line is open.

Q - Alice Nettleton

Hi, thank you for taking our questions. This is Alice Nettleton on for Tim Anderson. So a question on PBMs which are under renewed scrutiny if there were material changes to the rebating structure currently in place will that put the incumbent products at risk because it might remove the so-called rebate war? And more generally, do you think there is any chance of some of the proposed legislative changes actually becoming law?

And then secondly, any collateral impact you're seeing on Skyrizi or Rinvoq since Humira biosimilars has launched given the overlapping indications? Do you think that you'll start to see some picking of the patients from these two brands to biosimilars in the second half of the year? Thank you.

A - Jeff Stewart {[BIO 19485431 <GO>](#)}

Hi, it's Jeff. I'll give some comment on that. I think the way that we think about our brands is the first place that we look at is how distinctive they are. I mean, we've got four head-to-head trials with Skyrizi and another one on the way where we can clearly differentiate the product and we have many as well on Rinvoq.

So we've really thought about it from a development standpoint. And I would say the perspective is somehow, there was a restructuring of the PBMs, which I don't think is imminent and the rebate sort of approach disappeared. It disappears for everybody. I mean, all of these I&I products have a fairly reasonable rebate load and there'll be a different basis of competition, which we would do very very well. So we're not concerned about a fundamental structural change relative to these two products which are very distinctive.

If we look at your second part of your question which is, it's really the same answer, which is we don't see that they're going to be significant impacts of Humira biosimilars on the performance of Skyrizi and Rinvoq. And one perspective, let's take Rinvoq as a very simple way of thinking about it. It's already in the United States step behind TNF and it's performing at that level because you see such expansion of second and third lines in that space and Skyrizi is very very distinctive. So no, in the second half, we do not anticipate sort of a knock-on effect of the emergent biosimilars to our two core brands.

A - Liz Shea {[BIO 15134332](#) <GO>}

Thanks, Alice. Operator, next question please.

Operator

Thank you. Our next question comes from Carter Gould with Barclays. Your line is open.

Q - Carter Gould {[BIO 21330584](#) <GO>}

Great. Good morning, thanks for taking the question. Maybe a different spin on the question there just given the volatility in the marketplace as you have those conversations or engage with potential targets if you've seen a shift in that bid-ask spread and the willingness of boards and management to consider deals. Any updates on that front would be helpful.

A - Rick Gonzalez {[BIO 1973579](#) <GO>}

This is Rick. I'd say the environment hasn't changed materially in the last 24 months, from my perspective. I still think it's certainly more difficult to raise money for biotech company. So, it probably makes them a bit more willing to engage with players like us or engage in a process if they are at a point where they have generated data that makes them attractive.

But I'd say the interest level in that engagement is similar to what has been for the last 12 to 24 months and there's a lot of opportunities to be able to find assets that are in the biotech area. The question is, you have to find the right kind of asset and you have to buy one that's attractive and it meet your needs. I'd say being able to negotiate a transaction I think is a reasonable probability. I'd say prices are still relatively high and so valuations for good assets tend to go at a pretty high level. So again, it's got to be an asset that can demonstrate that it's going to provide significant value to justify that kind of valuation and a return. But we continue to look for opportunities and I think as we find those kinds of opportunities, as I've said in the past, we're certainly going to pursue them.

A - Liz Shea {[BIO 15134332](#) <GO>}

Thanks, Carter. Operator, next question please.

Operator

Our next question comes from Vamil Divan with Guggenheim Securities. Your line is open.

Q - Vamil Divan {[BIO 15748296](#) <GO>}

Great, thanks for taking the questions. Maybe a couple from me as well. So, one, just a couple of data points you have coming up this year that I think maybe a little bit less focused on is Navitoclax and Teliso-V, so maybe you can just sort of frame so what we should be looking for what your expectations are, what you're hoping to see from those assets especially Teliso-V, given your comments on the NextGen ABBV-400.

And then the other question, I guess would be for Rick and more on succession planning I know you've mentioned before that you plan to stick around through the Humira event. I'm just wondering if you have any updated thoughts on timing on that now that we're in the middle of this process, we've been getting some questions there. Is this something we should expect some announcement this year or is it more you're looking to 2024 or later? Thank you.

A - Roopal Thakkar

Hi, it's Roopal. I'll take the ones on Navitoclax and Teliso-V. So for Navitoclax, that's the combination with the JAK2 in myelofibrosis. And there, we're looking to be better than monotherapy with JAK2 in that space and improving spleen size and symptoms like abdominal pain, fullness, fever, fatigue. And also perhaps uniquely what we've observed is also an improvement in bone marrow fibrosis and a decrease and variant allelic frequency. So that's what we would be looking for and we should get a read-out by mid-year or so.

For Teliso-V in lung cancer EGFR wild-type with high c-MET and that's the indication, where we have Breakthrough Therapy Designation. Around the end of this year I would say that's where we would see a read-out. What we've seen in earlier data cuts is an ORR above 50% which is quite a bit higher than what would be expected in standard of care in that second, third-line setting. And if the data looks strong, there is a potential that we could submit next year for an accelerated approval.

A - Rick Gonzalez {BIO 1973579 <GO>}

I'll take the second question, and maybe I'll make a little bit of a comment on Teliso-V because I think you mentioned 400. I think the early data that we're seeing in 400 is impressive to us. There is no question about it. And I think we're going to have some data presented at this ASCO. Right? We will have an opportunity to see that in CRC. Now having said that Teliso-V as Roopal said does get very good responses in c-MET highs. But to get a broader set of c-MET population we do believe you need to move to the Topol warhead. It seems to get deeper responses and more durable responses. That data has to play out over time, but it appears to be a very good platform for c-MET.

And so we need the data to mature and we need to develop more data in that area, but I'd say the early data is pretty encouraging, you'll have a chance to see a snippet of that at this ASCO.

As far as leadership changes. I'd say it's similar to what we said in it's either the first quarter or fourth quarter, I can't recall last year, around succession. We obviously have a process in place, we have very experienced Board. I've had many, many discussions with the Board about succession. The Board knows I'm committed to be here to ensure a successful and smooth transition. The criteria that we're operating against is we need to completely get through the transition for Humira biosimilars here in the US.

I'd say so far I'm very pleased with how the transition is going, and I'm even more pleased with the way the growth plan platform is operating right now. And in fact, if you look at it this year, the growth platform is going to deliver mid-single-digits, and it's going to do it despite the headwind that we see on Imbruvica and the headwind that we're seeing from the economy on aesthetics. Once the aesthetics business returns to its normal growth rates and much of the pressure on Imbruvica starts to subside, we should see that growth rate increased significantly as we move into '24 for the growth platform. And obviously, we're turning to robust growth in '25 and deliver high-single-digit from that point forward. So that's the expectation that we're working against.

So we want to make sure that the business is operating the way we want it to, We want to make sure that we're through the biosimilar erosion to the point that we believe it is predictable and then obviously the second part of the criteria is ensuring that the candidate that will succeed me is ready to do that. We can make a successful transition. I've also told the Board that I'm willing to stay in any capacity that they would desire for whatever length of time they would. I think the expectation right now is that I would assume the Executive Chairman role for a period of time to

finish the transition to the new CEO. You should not be expecting that that transition is going to occur in '23.

A - Liz Shea {[BIO 15134332](#) <GO>}

Thanks, Vamil. Operator, next question, please.

Operator

Our next question comes from Evan Seigerman with BMO Capital Markets. Your line is open.

Q - Evan Seigerman {[BIO 18922817](#) <GO>}

Hi, guys, thank you so much for taking my question. Just on Skyrizi and some of the dynamics you're seeing there, can you characterize about how you're thinking about further penetration in the psoriasis indication, some puts and takes in the dermatology market? And just a follow-up on the CF program, is it safe to assume that you are totally done investing in this area or do you have other assets at an earlier development that could emerge? Thank you.

A - Jeff Stewart {[BIO 19485431](#) <GO>}

Hi, it's Jeff, I'll take your comment on psoriasis. I think that as I mentioned, the share is very, very impressive. So we have 30% total market share now, which is really putting significant headroom against any other drug in that category by a lot. And one way to think about it is I think what you're asking is how much further can that run and it can run quite a bit further to some degree, if you think about it.

So we're capturing on the dynamic share roughly 50%, so one out of every two patients and our market share is about 30. So theoretically, over time, unless there's some disruption, which we don't see significant disruption in the market, your total market share is going to move towards that in-play share. Now that takes many, many years, but as we look at the fundamental momentum that we can achieve it's still very, very significant.

Add on to that, basically, we're still and the rest of the world starting to really see the PSA ramp and remember PSA has a very significant impact because it's treated by derms and was the last remaining gap that we had. So you're going to see continued momentum in the international markets in the US market. And we have a long way to run. And I'd be remiss if I didn't say how fast again that we're growing in both Crohn's right now and very exciting data in UC and that market is very, very dynamic.

So we feel very secure in our ability to continue to create a lot of value with Skyrizi.

A - Tom Hudson {[BIO 15189431](#) <GO>}

Hi, this is Tom. Regarding the question on our triplet program. Our C2 corrector that we just tested did not work. This was not our first attempt at producing one and we do not have another backup. So we don't have other options than to discontinue the CF program.

Q - Evan Seigerman {[BIO 18922817](#) <GO>}

Great. Thank you.

A - Liz Shea {[BIO 15134332](#) <GO>}

Thanks, Evan. Operator, next question, please.

Operator

Thank you. Our next question comes from Trung win with Credit Suisse. Your line is open.

Q - Carson {BIO 7457306 <GO>}

Hi, good morning. This is Carson on for Trung. Thanks for the questions. Just on Imbruvica how confident are you for the 5.7 CMOG guide given the significant competitive pressure there? I mean, I understand, for teams that didn't particularly have leg in the US until late January. What level of pricing pressures can we expect through the year and if there is a potential for further step-down in your guide for later in the year as you did your last -- the trough you pushed out given the delay 951 as well. Thanks.

A - Jeff Stewart {BIO 19485431 <GO>}

Hi, it's Jeff. And I think we think that's a very good call. And just as a reminder, we're not seeing significant pricing pressure in the market. This is really two effects, which is one, the cumulative effect, as we've highlighted over the suppressed market over time, which looks to be normalizing. Actually, for the first time in three years, we actually saw a positive growth in the market. So that's encouraging. The big driver is the share, is the patient share which has been under pressure. Initially under pressure by Calquence certainly from our own Venclexta and then the recent Brukinsa launch.

And so when we put all of that into the calculus we think we've got it right, and it's probably unlikely that we're going to see any significant step-down that would put that in jeopardy.

A - Rob Michael {BIO 19541407 <GO>}

Carson, this is Rob, I'll answer your second question. So I wouldn't consider Imbruvica 951 to be variables that would push the trough out. It's really more about how the overall year plays out, particularly the second half with US Humira. So if US Humira does better and we outperform in '23 then we could see the trough in '24. I think the important thing to keep in mind is, regardless of when the trough occurs, we wouldn't expect earnings to fall below the 1074 ex IPR&D, that's really what I would focus on. And we don't consider Imbruvica 951 delayed to be variables that would push that trough.

A - Liz Shea {BIO 15134332 <GO>}

Thanks, Carson. We are cognizant of a number of peers reporting today and so in the interest of time we have time for one last question.

Operator

Our last question comes from Geoff Meacham with Bank of America. Your line is open.

Q - Susan {BIO 15127379 <GO>}

Hi, this is Susan on for Geoff Meacham. We had a follow-up on Imbruvica. Do you guys expect any changes to outlook following MCL/MDL withdrawal? And then do you expect any read-through to follicular lymphoma from that withdrawal?

A - Jeff Stewart {BIO 19485431 <GO>}

Hi, it's Jeff. Thank you for the question. First, these are very small indications. So to give you some sense of the relative size for Imbruvica MCL is about 4% of the value. MCL is really less than a percent, about 1%. So we don't anticipate that those withdrawals, due to the fact that we didn't get the confirmatory studies to clear will have a material impact.

I think it's also important to note that. Many physicians will continue these patients on the medication. They won't be all switched, for example, or taken off and put on another product, that's the market intelligence there is no requirement that they need to do that for the physicians. And so net-net, this is not really material issue given the size of those indications and I think Roopal will address your point on follicular.

A - Roopal Thakkar

I can talk about FL here for a minute. So a phase III readout is expected later this year. It's not clear if the MCL outcomes would be reflected in what we see there. But what. I would say about FL is our focus would be with Epcoritamab our dual engager which we expect DLBCL action here soon and then we have programs and we're seeing very-high levels of response in FL with Epcor either as a monotherapy or in combinations, which we'll see some data in those combos in DLBCL and FL at ASCO as well.

A - Liz Shea {[BIO 15134332 <GO>](#)}

Thanks, Susan. And that concludes today's conference call. If you'd like to listen to a replay of the call, please visit our website at investors.abbvie.com. Thanks again for joining us.

Operator

Thank you for your participation. Participants you may disconnect at this time.

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